

Emergency Services and Urgent Care Coverage Policy 2026

Effective January 1, 2026 | Confidential · For Internal Use

1. Emergency Service Definition

Emergency medical services are defined as services required for conditions that could reasonably be expected to place the member's health in serious jeopardy, result in serious impairment to bodily functions, or result in serious dysfunction of any bodily organ or part. This definition follows the prudent layperson standard and applies regardless of the final diagnosis.

2. Emergency Department Coverage

Emergency department (ED) visits are covered for all plan types without prior authorization. CPT codes 99281-99285 for ED evaluation and management are covered. Members are not required to obtain referrals or authorization for emergency care. However, follow-up outpatient care after the emergency may require prior authorization if the service is normally subject to prior auth requirements.

3. Retrospective Authorization for Emergency Services

When a member receives emergency care at an out-of-network facility, retrospective authorization is required within 24 hours of admission for inpatient stays following emergency admission. The previous 48-hour window has been reduced to 24 hours effective January 1, 2026. Claims for emergency services must include emergency diagnosis codes (ICD-10 codes beginning with S, T, or appropriate Z codes).

4. Observation Services

Hospital observation status (CPT 99218-99220, 99234-99236) is covered when ordered by an attending physician. Observation stays are billed as outpatient services. Members in observation status are responsible for outpatient cost-sharing, not inpatient cost-sharing. Observation stays exceeding 48 hours require clinical review and potential conversion to inpatient status.

5. Ambulance Services

Emergency ambulance transportation is covered when medically necessary and transport was to the nearest appropriate facility. Ground ambulance (A0427-A0434) and air ambulance (A0431, A0436) are covered for emergencies. Non-emergency ambulance transport requires prior authorization. Balance billing protection applies: members are protected from surprise billing for ambulance services.

6. Out-of-Network Emergency Protections

Federal No Surprises Act protections apply: members are protected from out-of-network cost-sharing for emergency services at out-of-network facilities. Members pay in-network cost-sharing amounts. Providers are prohibited from balance billing for emergency services. Independent dispute resolution (IDR) process resolves payment disputes between payers and out-of-network providers.

7. Mental Health Emergency Services

Psychiatric emergencies are treated the same as medical emergencies. Crisis intervention services (CPT 90839, 90840) are covered without prior authorization. Mobile crisis teams are covered under expanded behavioral health benefits. Involuntary psychiatric holds are covered with retrospective notification within 24 hours.